

Refer To

- **THSC (recommended)** for coordinated triage
- Or nominate a **specific specialist** if preferred

No definitive diagnosis required.

Send Referrals

HealthLink:	uticlini
Fax:	08 6555 6578
Email:	emailus@thsc.au
Phone:	08 6555 6558

When to Refer

- Recurrent UTIs
- Persistent LUTS
- Incontinence (bladder / bowel)
- Prostate symptoms
- Prolapse
- Pelvic pain
- Diagnostic uncertainty

Please Include (if available)

- MSU / urinalysis
- PSA (if relevant)
- Imaging (US / CT / MRI / proctography)
- Bladder diary (3-day preferred) / bowel chart
- Medications and prior treatments
- Operative history (including mesh)

Referral should not be delayed if incomplete.

Patient Self-Referral

No GP referral needed for:

- Specialist Nursing
- Pelvic Health Physiotherapy
- Clinical Psychology
- Pharmacy

Initial Assessment

Most patients are reviewed first by a **Specialist Nurse**, providing:

- Structured triage and symptom mapping
- Early investigation planning
- Direct allocation to appropriate specialist(s)

Communication

- Clinical notes available on request
- Complex cases: **single consolidated letter after MDT**
- Avoids fragmented or interim correspondence

Care Model

- Single coordinated pathway
- Multidisciplinary care as required
- Avoids repeated tests and fragmented referrals
- Diagnosis-led management

GP Care Plans

CDM / EPC plans and Team Care Arrangements recommended for patients engaging with:

- Pelvic Health Physiotherapy
- Clinical Psychology
- Pain Medicine

Red Flags (Urgent)

Expedite referral

Visible haematuria · Suspected malignancy (pelvic mass, raised PSA)

Urgent ED referral

Acute urinary retention · Sepsis · New neurological deficit (cauda equina) · Acute severe pelvic pain

Hours: Mon–Fri 9:00am – 4:00pm · Closed weekends & public holidays